

UNIDEX Dexmedetomidine Injection USP Concentrate for Infusion 100 mcg/mL

COMPOSITION:

Each vial contains: -

Dexmedetomidine Hydrochloride USP..... 118mcg

Sodium Chloride USP.....9.0 mg

Water for Injection BP.....q.s

DOSAGE FORM

Concentrate for infusion

PRODUCT DESCRIPTION

UNIDEX- Dexmedetomidine Injection USP Concentrate for Infusion 100 mcg/ml is Clear, colorless solution, free from visible extraneous matter.

Each vial contains:- Dexmedetomidine Hydrochloride USP..... 118mcg

Description of product after dilution: A Clear colorless solution.

Excipients with known effects:

Each vial of 2ml contains 7.08mg of sodium.

PHARMACODYNAMICS

In a study in healthy volunteers (N=10), respiratory rate and oxygen saturation remained within normal limits and there was no evidence of respiratory depression when dexmedetomidine was administered by intravenous infusion at doses within the recommended dose range (0.2 - 0.7 mcg/kg/hr).

PHARMACOKINETICS

Following intravenous administration, dexmedetomidine exhibits the following pharmacokinetic parameters: a rapid distribution phase with a distribution half-life ($t_{1/2}$) of approximately 6 minutes; a terminal elimination half-life ($t_{1/2}$) of approximately 2 hours; and steady-state volume of distribution (V_{ss}) of approximately 118 liters. Clearance is estimated to be approximately 39 L/h. The mean body weight associated with this clearance estimate was 72 kg.

Dexmedetomidine exhibits linear pharmacokinetics in the dosage range of 0.2 to 0.7 mcg/kg/hr when administered by intravenous infusion for up to 24 hours. Below table shows the main pharmacokinetic parameters when Dexmedetomidine was infused (after appropriate loading doses) at maintenance infusion rates of 0.17 mcg/kg/hr (target plasma concentration of 0.3 ng/mL) for 12 and 24 hours, 0.33 mcg/kg/hr (target plasma concentration of 0.6 ng/mL) for 24 hours, and 0.70 mcg/kg/hr (target plasma concentration of 1.25 ng/mL) for 24 hours.

Table 8. Mean ± SD Pharmacokinetic Parameters				
Parameter	Loading Infusion (min)/Total infusion duration (hrs)			
	10 min/12 hrs	10 min/24 hrs	10 min/24 hrs	35 min/24 hrs
	Precedex Target Plasma Concentration (ng/mL) and Dose (mcg/kg/hr)			
	0.3/0.17	0.3/0.17	0.6/0.33	1.25/0.70
$t_{1/2}^*$, hour	1.78 ± 0.30	2.22 ± 0.59	2.23 ± 0.21	2.50 ± 0.61
CL, liter/hour	46.3 ± 8.3	43.1 ± 6.5	35.3 ± 6.8	36.5 ± 7.5
V_{ss} , liter	88.7 ± 22.9	102.4 ± 20.3	93.6 ± 17.0	99.6 ± 17.8
Avg $C_{ss}^{\#}$, ng/mL	0.27 ± 0.05	0.27 ± 0.05	0.67 ± 0.10	1.37 ± 0.20

* Presented as harmonic mean and pseudo standard deviation.

[#] Mean C_{ss} = Average steady-state concentration of Dexmedetomidine. The mean C_{ss} was calculated based on post-dose sampling from 2.5 - 9 hours samples for 12 hour infusion and post-dose sampling from 2.5 - 18 hours for 24 hour infusions.

The loading doses for each of the above indicated groups were 0.5, 0.5, 1 and 2.2 mcg/kg, respectively.

Dexmedetomidine pharmacokinetic parameters after Dexmedetomidine maintenance doses of 0.2 to 1.4 mcg/kg/hr for >24 hours were similar to the PK parameters after Dexmedetomidine maintenance dosing for <24 hours in other studies. The values for clearance (CL), volume of distribution (V), and t_{1/2} were 39.4 L/hr, 152 L, and 2.67 hours, respectively.

Distribution

The steady-state volume of distribution (V_{ss}) of dexmedetomidine was approximately 118 liters. Dexmedetomidine protein binding was assessed in the plasma of normal healthy male and female subjects. The average protein binding was 94% and was constant across the different plasma concentrations tested. Protein binding was similar in males and females. The fraction of Dexmedetomidine injection that was bound to plasma proteins was significantly decreased in subjects with hepatic impairment compared to healthy subjects.

The potential for protein binding displacement of dexmedetomidine by fentanyl, ketorolac, theophylline, digoxin and lidocaine was explored in vitro, and negligible changes in the plasma protein binding of Dexmedetomidine were observed. The potential for protein binding displacement of phenytoin, warfarin, ibuprofen, propranolol, theophylline and digoxin by Dexmedetomidine was explored in vitro and none of these compounds appeared to be significantly displaced by Dexmedetomidine.

Metabolism

Dexmedetomidine undergoes almost complete biotransformation with very little unchanged dexmedetomidine excreted in urine and feces. Biotransformation involves both direct glucuronidation as well as cytochrome P450 mediated metabolism. The major metabolic pathways of dexmedetomidine are: direct N-glucuronidation to inactive metabolites; aliphatic hydroxylation (mediated primarily by CYP2A6 with a minor role of CYP1A2, CYP2E1, CYP2D6 and CYP2C19) of dexmedetomidine to generate 3- hydroxydexmedetomidine, the glucuronide of 3- hydroxy- dexmedetomidine, and 3-carboxy dexmedetomidine; and N methylation of dexmedetomidine to generate 3-hydroxy N-methyl- dexmedetomidine, 3-carboxy N-methyl- dexmedetomidine, and dexmedetomidine-N-methyl O-glucuronide.

Elimination

The terminal elimination half-life (t_{1/2}) of dexmedetomidine is approximately 2 hours and clearance is estimated to be approximately 39 L/h. A mass balance study demonstrated that after nine days an average of 95% of the radioactivity, following intravenous administration of radiolabeled dexmedetomidine, was recovered in the urine and 4% in the feces. No unchanged dexmedetomidine was detected in the urine. Approximately 85% of the radioactivity recovered in the urine was excreted within 24 hours after the infusion. Fractionation of the radioactivity excreted in urine demonstrated that products of N-glucuronidation accounted for approximately 34% of the cumulative urinary excretion. In addition, aliphatic hydroxylation of parent drug to form 3-hydroxydexmedetomidine, 3-hydroxy- dexmedetomidine, the glucuronide of 3-hydroxy- dexmedetomidine, and 3-carboxylic acid-dexmedetomidine together represented approximately 14% of the dose in urine. N-methylation of dexmedetomidine to form 3 hydroxy N-methyl dexmedetomidine, 3-carboxy N-methyl dexmedetomidine, and N methyl O glucuronide dexmedetomidine accounted for approximately 18% of the dose in urine. The N Methyl metabolite itself was a minor circulating component and was undetected in urine. Approximately 28% of the urinary metabolites have not been identified.

Gender:

There was no observed difference in Dexmedetomidine injection pharmacokinetics due to gender.

Geriatrics:

The pharmacokinetic profile of Dexmedetomidine injection was not altered by age. There were no differences in the pharmacokinetics of Dexmedetomidine injection in young (18 - 40 years), middle age (41 - 65 years), and elderly (>65 years) subjects.

Hepatic Impairment:

In subjects with varying degrees of hepatic impairment (Child-Pugh Class A, B, or C), clearance values for Dexmedetomidine injection were lower than in healthy subjects. The mean clearance values for patients with mild, moderate, and severe hepatic impairment were 74%, 64% and 53% of those observed in the normal healthy subjects, respectively. Mean clearances for free drug were 59%, 51% and 32% of those observed in the normal healthy subjects, respectively.

Although Dexmedetomidine injection is dosed to effect, it may be necessary to consider dose reduction in subjects with hepatic impairment.

Renal Impairment:

Dexmedetomidine injection pharmacokinetics (C_{max} , T_{max} , AUC, $t_{1/2}$, CL, and V_{ss}) were not significantly different in patients with severe renal impairment (creatinine clearance: <30 mL/min) compared to healthy subjects.

INDICATION

1.1. Intensive Care Unit Sedation

UNIDEX is indicated for sedation of initially intubated and mechanically ventilated patients during treatment in an intensive care setting.

UNIDEX should be administered by continuous infusion not to exceed 24 hours.

UNIDEX has been continuously infused in mechanically ventilated patients prior to extubation, during extubation, and post-extubation. It is not necessary to discontinue Dexmedetomidine injection prior to extubation.

1.2. Procedural Sedation

UNIDEX is indicated for sedation of non-intubated patients prior to and/or during surgical and other procedures.

RECOMMENDED DOSE

2.1 Dosing Guidelines

- UNIDEX dosing should be individualized and titrated to desired clinical response.
- UNIDEX is not indicated for infusions lasting longer than 24 hours
- UNIDEX should be administered using a controlled infusion device.

2.2 Dosage Information

Table 1. Dosage Information

INDICATION	DOSAGE AND ADMINISTRATION
Initiation of Intensive Care Unit Sedation:	For adult patients: a loading infusion of one mcg/kg over 10 minutes. For patients over 65 years of age: a dose reduction should be considered. For patients with impaired hepatic or renal function: a dose reduction should be considered.
Maintenance of Intensive Care Unit Sedation:	For adult patients: a maintenance infusion of 0.2 to 0.7 mcg/kg/hr. The rate of the maintenance infusion should be adjusted to achieve the desired level of sedation. For patients over 65 years of age: a dose reduction should be considered. For patients with impaired hepatic or renal function: a dose reduction should be considered.
Initiation of Procedural Sedation:	For adult patients: a loading infusion of one mcg/kg over 10 minutes. For less invasive procedures such as ophthalmic surgery, a loading infusion of 0.5 mcg/kg given over 10 minutes may be suitable. For awake fiberoptic intubation patients: a loading infusion of one mcg/kg over 10 minutes. For patients over 65 years of age: a loading infusion of 0.5 mcg/kg over 10 minutes. For patients with impaired hepatic or renal function: a dose reduction should be considered.
Maintenance of Procedural Sedation:	For adult patients: the maintenance infusion is generally initiated at 0.6 mcg/kg/hr and titrated to achieve desired clinical effect with doses ranging from 0.2 to 1 mcg/kg/hr. The rate of the maintenance infusion should be adjusted to achieve the targeted level of sedation. For awake fiberoptic intubation patients: a maintenance infusion of 0.7 mcg/kg/hr is recommended until the endotracheal tube is secured. For patients over 65 years of age: a dose reduction should be considered. For patients with impaired hepatic or renal function: a dose reduction should be considered.

2.3. Dosage Adjustment

Due to possible pharmacodynamic interactions, a reduction in dosage of UNIDEX or other concomitant anesthetics, sedatives, hypnotics or opioids may be required when co-administered.

Dosage reductions may need to be considered for patients with renal or hepatic impairment, and geriatric patients.

2.4. Preparation of Solution

UNIDEX must be diluted in 0.9% sodium chloride solution to achieve required concentration (4 mcg/mL) prior to administration. Preparation of solutions is the same, whether for the loading dose or maintenance infusion.

Strict aseptic technique must always be maintained during handling of UNIDEX.

To prepare the infusion, withdraw 2 mL of UNIDEX and add to 48 mL of 0.9% sodium chloride injection to a total of 50 mL. Shake gently to mix well.

Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit.

2.5. Administration with Other Fluids

UNIDEX infusion should not be co-administered through the same intravenous catheter with blood or plasma because physical compatibility has not been established.

UNIDEX has been shown to be incompatible when administered with the following drugs: amphotericin B, diazepam.

UNIDEX has been shown to be compatible when administered with 0.9% sodium chloride injection only.

ROUTE OF ADMINISTRATION

Intravenous Infusion

CONTRAINDICATION

Patients who are hypersensitive to this drug or to any ingredient in the formulation or component of the container

WARNING AND PRECAUTIONS

5.1. Drug Administration

Dexmedetomidine injection should be administered only by persons skilled in the management of patients in the intensive care or operating room setting. Due to the known pharmacological effects of Dexmedetomidine injection, patients should be continuously monitored while receiving Dexmedetomidine injection.

5.2. Hypotension, Bradycardia, and Sinus Arrest

Clinically significant episodes of bradycardia and sinus arrest have been reported with Dexmedetomidine injection administration in young, healthy adult volunteers with high vagal tone or with different routes of administration including rapid intravenous or bolus administration. Reports of hypotension and bradycardia have been associated with Dexmedetomidine injection infusion. Some of these cases have resulted in fatalities. If medical intervention is required, treatment may include decreasing or stopping the infusion of Dexmedetomidine injection, increasing the rate of intravenous fluid administration, elevation of the lower extremities, and use of pressor agents. Because Dexmedetomidine injection has the potential to augment bradycardia induced by vagal stimuli, clinicians should be prepared to intervene. The intravenous administration of anticholinergic agents (e.g., glycopyrrolate, atropine) should be considered to modify vagal tone. In clinical trials, glycopyrrolate or atropine were effective in the treatment of most episodes of Dexmedetomidine injection-induced bradycardia. However, in some patients with significant cardiovascular dysfunction, more advanced resuscitative measures were required.

Caution should be exercised when administering Dexmedetomidine injection to patients with advanced heart block and/or severe ventricular dysfunction. Because Dexmedetomidine injection decreases sympathetic nervous system activity, hypotension and/or bradycardia may be expected to be more pronounced in patients with hypovolemia, diabetes mellitus, or chronic hypertension and in elderly patients.

5.3. Transient Hypertension

Transient hypertension has been observed primarily during the loading dose in association with the initial peripheral vasoconstrictive effects of Dexmedetomidine injection. Treatment of the transient hypertension has generally not been necessary, although reduction of the loading infusion rate may be desirable.

5.4. Arousability

Some patients receiving Dexmedetomidine injection have been observed to be arousable and alert when stimulated. This alone should not be considered as evidence of lack of efficacy in the absence of other clinical signs and symptoms.

5.5. Withdrawal

Intensive Care Unit Sedation

The most common events were nausea, vomiting, and agitation.

If tachycardia and/or hypertension occurs after discontinuation of Dexmedetomidine injection supportive therapy is indicated.

Procedural Sedation

Withdrawal symptoms were not seen after discontinuation of short term infusions of Dexmedetomidine injection (<6 hours).

5.6 Tolerance and Tachyphylaxis

Use of dexmedetomidine beyond 24 hours has been associated with tolerance and tachyphylaxis and a dose-related increase in adverse reactions.

5.7 Hyperthermia or Pyrexia

Dexmedetomidine injection may induce hyperthermia or pyrexia, which may be resistant to traditional cooling methods, such as administration of cooled intravenous fluids and antipyretic medications. Discontinue Dexmedetomidine injection if drug-related hyperthermia or pyrexia is suspected and monitor patients until body temperature normalizes.

5.8 Hepatic Impairment

Since Dexmedetomidine injection clearance decreases with severity of hepatic impairment, dose reduction should be considered in patients with impaired hepatic function.

5.9 Incompatibilities

Dexmedetomidine injection infusion should not be co-administered through the same intravenous catheter with blood, serum, or plasma because physical compatibility has not been established.

Dexmedetomidine injection has been shown to be incompatible when administered with the following drugs: amphotericin B, diazepam.

INTERACTIONS WITH OTHER MEDICAMENTS

Anesthetics, Sedatives, Hypnotics, Opioids

Co-administration of Dexmedetomidine injection with anesthetics, sedatives, hypnotics, and opioids is likely to lead to an enhancement of effects. Specific studies have confirmed these effects with sevoflurane, isoflurane, propofol, alfentanil, and midazolam. No pharmacokinetic interactions between Dexmedetomidine injection and isoflurane, propofol, alfentanil and midazolam have been demonstrated. However, due to possible pharmacodynamic interactions, when co-administered with Dexmedetomidine injection, a reduction in dosage of Dexmedetomidine injection or the concomitant anesthetic, sedative, hypnotic or opioid may be required.

Neuromuscular Blockers

Administration of Dexmedetomidine injection for 45 minutes at a plasma concentration of one ng/mL resulted in no clinically meaningful increases in the magnitude of neuromuscular blockade associated with rocuronium administration.

PREGNANCY AND LACTATION

Risk Summary

Available data from published randomized controlled trials and case reports over several decades of use with intravenously administered dexmedetomidine during pregnancy have not identified a drug associated risk of major birth defects and miscarriage; however, the reported exposures occurred after the first trimester. Most of the available data are based on studies with exposures that occurred at the time of caesarean section delivery, and these studies have not identified an adverse effect on maternal outcomes or infant Apgar scores. Available data indicate that dexmedetomidine crosses the placenta. In animal reproduction studies, fetal toxicity that lower fetal viability and reduced live fetuses occurred with subcutaneous administration of dexmedetomidine to pregnant rats during organogenesis at doses 1.8 times the maximum recommended human dose (MRHD) of 17.8mcg/kg/day. Developmental toxicity (low pup weights and adult offspring weights, decreased F1 grip strength, increased early implantation loss and decreased viability of second-generation offspring) occurred when pregnant rats were subcutaneously administered dexmedetomidine at doses less than the clinical dose from late pregnancy through lactation and weaning.

The estimated background risk of major birth defects and miscarriage for the indicated population is unknown. All pregnancies have a background risk of birth defect, loss, or other adverse outcomes. In the U.S. general population, the estimated background risk of major birth defects and miscarriage in clinically recognized pregnancies is 2-4% and 15-20%, respectively.

Data

Animal Data

Increased post-implantation losses and reduced live fetuses in the presence of maternal toxicity (i.e. decreased body weight) were noted in a rat embryo-fetal development study in which pregnant dams were administered subcutaneous doses of dexmedetomidine 200 mcg/kg/day (equivalent to 1.8 times the intravenous MRHD of 17.8 mcg/kg/day based on body surface area [BSA]) during the period of organogenesis (Gestation Day [GD] 6 to 15). No malformations were reported. No malformations or embryo-fetal toxicity were noted in a rabbit embryo-fetal development study in which pregnant does were administered dexmedetomidine intravenously at doses of up to 96 mcg/kg/day (approximately half the human exposure at the MRHD based on AUC) during the period of organogenesis (GD6 to 18). Reduced pup and adult offspring birth weights, and grip strength were reported in a rat developmental toxicology study in which pregnant females were administered dexmedetomidine subcutaneously at doses of 8mcg/kg/day (0.07 times the MRHD based on BSA) during late pregnancy through lactation and weaning (GD16 to postnatal day [PND] 25). Decreased viability of second generation offspring and an increase in early implantation loss along with delayed motor development occurred in the 32mcg/kg/day group (equivalent to less than the clinical dose based on BSA) when first generation offspring were allowed to mate. This study limited dosing to hard palate closure (GD 15 to 18) through weaning instead of dosing from implantation (GD6 to 7) to weaning (PND21). In a study in the pregnant rat, placental transfer of dexmedetomidine was observed when radio labeled dexmedetomidine was administered subcutaneously.

Lactation

Risk Summary

Available published literature reports the presence of dexmedetomidine in human milk following intravenous administration. There is no information regarding the effects of dexmedetomidine on the breastfed infant or the effects on milk production. Advise women to monitor the breastfed infant for irritability. The developmental and health benefits of breastfeeding should be considered along with the mother's clinical need for Dexmedetomidine injection and any potential adverse effects on the breast fed infant from Dexmedetomidine injection or from the underlying condition. Data In two published clinical studies, a total of 14 women were given intravenous dexmedetomidine 6mcg/kg/hour for 10 minutes after delivery followed by continuous infusion of 0.2–0.7 mcg/kg/hour. Breast milk and maternal blood samples were collected at 0, 6, 12, and 24 hours after discontinuation of dexmedetomidine. Plasma and milk dexmedetomidine concentrations were detectable up to 6 hours in most subjects, up to 12 hours in one subject and undetectable in all at 24 hours. The milk-to-plasma ratio from single paired maternal milk and plasma concentrations at each time point ranged from 0.53 to 0.95. The relative infant dose was estimated to range from 0.02 to 0.098%.

SIDE EFFECTS

The following clinically significant adverse reactions are described elsewhere in the labeling: Hypotension, bradycardia and sinus arrest. Transient hypertension.

Clinical Trials Experience

Because clinical trials are conducted under widely varying conditions, adverse reactions rates observed in the clinical trials of a drug cannot be directly compared to rates in clinical trials of another drug and may not reflect the rates observed in practice. Most common treatment-emergent adverse reactions, occurring in greater than 2% of patients in both Intensive Care Unit and procedural sedation studies include hypotension, bradycardia and dry mouth.

Intensive Care Unit Sedation

Adverse reaction information is derived from the continuous infusion trials of Dexmedetomidine injection for sedation in the Intensive Care Unit setting in which 1007 adult patients received Dexmedetomidine injection. The mean total dose was 7.4 mcg/kg (range: 0.8 to 84.1), mean dose per hour was 0.5 mcg/kg/hr (range: 0.1 to 6.0) and the mean duration of infusion of 15.9 hours (range: 0.2 to 157.2). The population was between 17 to 88 years of age, 43% ≥65 years of age, 77% male and 93% Caucasian. Treatment-emergent adverse reactions occurring at an incidence of >2% are provided in Table 2. The most frequent adverse reactions were hypotension, bradycardia and dry mouth.

Adverse Reactions with an Incidence > 2%- Adult Intensive Care Unit Sedation Population <24 hours*

Adverse Event	All Dexmedetomidine injection (N = 1007) (%)	Randomized Dexmedetomidine injection (N = 798) (%)	Placebo (N = 400) (%)	Propofol (N = 188) (%)
Hypotension	25%	24%	12%	13%
Hypertension	12%	13%	19%	4%
Nausea	9%	9%	9%	11%
Bradycardia	5%	5%	3%	0
Atrial Fibrillation	4%	5%	3%	7%
Pyrexia	4%	4%	4%	4%
Dry Mouth	4%	3%	1%	1%
Vomiting	3%	3%	5%	3%
Hypovolemia	3%	3%	2%	5%
Atelectasis	3%	3%	3%	6%
Pleural Effusion	2%	2%	1%	6%
Agitation	2%	2%	3%	1%
Tachycardia	2%	2%	4%	1%
Anemia	2%	2%	2%	2%
Hyperthermia	2%	2%	3%	0
Chills	2%	2%	3%	2%
Hyperglycemia	2%	2%	2%	3%
Hypoxia	2%	2%	2%	3%
Post-procedural Hemorrhage	2%	2%	3%	4%
Pulmonary Edema	1%	1%	1%	3%
Hypocalcemia	1%	1%	0	2%
Acidosis	1%	1%	1%	2%
Urine Output Decreased	1%	1%	0	2%
Sinus Tachycardia	1%	1%	1%	2%
Ventricular Tachycardia	<1%	1%	1%	5%
Wheezing	<1%	1%	0	2%
Edema Peripheral	<1%	0	1%	2%

* 26 subjects in the all Dexmedetomidine injection group and 10 subjects in the randomized Dexmedetomidine injection group had exposure for greater than 24 hours.

Adverse reaction information was also derived from the placebo-controlled, continuous infusion trials of Dexmedetomidine injection for sedation in the surgical intensive care unit setting in which 387 adult patients received Dexmedetomidine injection for less than 24 hours. The most frequently observed treatment-emergent adverse events included hypotension, hypertension, nausea, bradycardia, fever, vomiting, hypoxia, tachycardia and anemia (See Table 3)

Table 3: Treatment-Emergent Adverse Events Occurring in >1% of All Dexmedetomidine-Treated Adult Patients in the Randomized Placebo-Controlled Continuous Infusion <24 Hours ICU Sedation Studies

Adverse Event	Randomized Dexmedetomidine (N = 387)	Placebo (N = 379)
Hypotension	28%	13%
Hypertension	16%	18%
Nausea	11%	9%
Bradycardia	7%	3%
Fever	5%	4%
Vomiting	4%	6%
Atrial Fibrillation	4%	3%
Hypoxia	4%	4%
Tachycardia	3%	5%
Hemorrhage	3%	4%
Anemia	3%	2%
Dry Mouth	3%	1%
Rigors	2%	3%
Agitation	2%	3%
Hyperpyrexia	2%	3%
Pain	2%	2%
Hyperglycemia	2%	2%

Acidosis	2%	2%
Pleural Effusion	2%	1%
Oliguria	2%	<1%
Thirst	2%	<1%

In a controlled clinical trial, Dexmedetomidine injection was compared to midazolam for ICU sedation exceeding 24 hours duration in adult patients. Key treatment emergent adverse events occurring in dexmedetomidine or midazolam treated patients in the randomized active comparator continuous infusion long-term intensive care unit sedation study are provided in Table 4. The number (%) of subjects who had a dose-related increase in treatment-emergent adverse events by maintenance adjusted dose rate range in the Dexmedetomidine injection group is provided in Table 5.

Table 4: Key Treatment-Emergent Adverse Events Occurring in Dexmedetomidine- or Midazolam-Treated Adult Patients in the Randomized Active Comparator Continuous Infusion Long-Term Intensive Care Unit Sedation Study

Adverse Event	Dexmedetomidine (N = 244)	Midazolam (N = 122)
Hypotension ¹	56%	56%
Hypotension Requiring Intervention	28%	27%
Bradycardia ²	42%	19%
Bradycardia Requiring Intervention	5%	1%
Systolic Hypertension ³	28%	42%
Tachycardia ⁴	25%	44%
Tachycardia Requiring Intervention	10%	10%
Diastolic Hypertension ³	12%	15%
Hypertension ³	11%	15%
Hypertension Requiring Intervention [†]	19%	30%
Hypokalemia	9%	13%
Pyrexia	7%	2%
Agitation	7%	6%
Hyperglycemia	7%	2%
Constipation	6%	6%
Hypoglycemia	5%	6%
Respiratory Failure	5%	3%
Renal Failure Acute	2%	1%
Acute Respiratory Distress Syndrome	2%	1%
Generalized Edema	2%	6%
Hypomagnesemia	1%	7%

[†] Includes any type of hypertension.

- ¹ Hypotension was defined in absolute terms as Systolic blood pressure of <80 mmHg or Diastolic blood pressure of <50 mmHg or in relative terms as ≤30% lower than pre-study drug infusion value.
- ² Bradycardia was defined in absolute terms as <40 bpm or in relative terms as ≤30% lower than pre-study drug infusion value.
- ³ Hypertension was defined in absolute terms as Systolic blood pressure >180 mmHg or Diastolic blood pressure of >100 mmHg or in relative terms as ≥30% higher than pre-study drug infusion value.
- ⁴ Tachycardia was defined in absolute terms as >120 bpm or in relative terms as ≥30% greater than pre-study drug infusion value.

The following adverse events occurred between 2 and 5% for Dexmedetomidine injection and Midazolam, respectively: renal failure acute (2.5%, 0.8%), acute respiratory distress syndrome (2.5%, 0.8%), and respiratory failure (4.5%, 3.3%).

Table 5. Number (%) of Adult Subjects Who Had a Dose-Related Increase in Treatment Emergent Adverse Events by Maintenance Adjusted Dose Rate Range in the Dexmedetomidine injection Group

Adverse Event	Dexmedetomidine injection mcg/kg/hr		
	≤0.7* (N = 95)	>0.7 to ≤1.1* (N = 78)	>1.1* (N = 71)
Constipation	6%	5%	14%
Agitation	5%	8%	14%

Anxiety	5%	5%	9%
Edema Peripheral	3%	5%	7%
Atrial Fibrillation	2%	4%	9%
Respiratory Failure	2%	6%	10%
Acute Respiratory Distress Syndrome	1%	3%	9%

* Average maintenance dose over the entire study drug administration

Procedural Sedation

Adverse reaction information is derived from the two trials for procedural sedation in which 318 adult patients received Dexmedetomidine Injection. The mean total dose was 1.6 mcg/kg (range: 0.5 to 6.7), mean dose per hour was 1.3 mcg/kg/hr (range: 0.3 to 6.1) and the mean duration of infusion of 1.5 hours (range: 0.1 to 6.2). The population was between 18 to 93 years of age, ASA I-IV, 30% ≥65 years of age, 52% male and 61% Caucasian.

Treatment-emergent adverse reactions occurring at an incidence of >2% are provided in Table 6. The most frequent adverse reactions were hypotension, bradycardia, and dry mouth. Pre-specified criteria for the vital signs to be reported as adverse reactions are footnoted below the table. The decrease in respiratory rate and hypoxia was similar between Dexmedetomidine Injection and comparator groups in both studies.

Table 6. Adverse Reactions With an Incidence > 2%— Procedural Sedation Population

Adverse Event	Dexmedetomidine injection N = 318	Placebo N = 113
	n (%)	n (%)
Hypotension ¹	54%	30%
Respiratory Depression ²	37%	32%
Bradycardia ³	14%	4%
Hypertension ⁴	13%	24%
Tachycardia ⁵	5%	17%
Nausea	3%	2%
Dry Mouth	3%	1%
Hypoxia ⁶	2%	3%
Bradypnea	2%	4%

¹ Hypotension was defined in absolute and relative terms as Systolic blood pressure of <80 mmHg or ≤30% lower than pre- study drug infusion value, or diastolic blood pressure of <50 mmHg

² Respiratory depression was defined in absolute and relative terms as respiratory rate (RR)<8 beats per minute or >25% decrease from baseline

³ Bradycardia was defined in absolute and relative terms as <40 beats per minute or ≤30% lower than pre-study drug infusion value.

⁴ Hypertension was defined in absolute and relative terms as Systolic blood pressure >180 mmHg or ≥30% higher than pre- study drug infusion value or diastolic blood pressure of >100 mmHg.

⁵ Tachycardia was defined in absolute and relative terms as >120 beats per minute or ≥30% greater than pre-study drug infusion value.

⁶ Hypoxia was defined in absolute and relative terms as SpO₂ < 90% or 10% decrease from baseline

Postmarketing Experience

The following adverse reactions have been identified during post approval use of Dexmedetomidine injection. Because these reactions are reported voluntarily from a population of uncertain size, it is not always possible to reliably estimate their frequency or establish a causal relationship to drug exposure.

Hypotension and bradycardia were the most common adverse reactions associated with the use of Dexmedetomidine injection during post approval use of the drug.

Table 7: Adverse Reactions Experienced During Post Approval Use of Dexmedetomidine injection	
System Organ Class	Preferred Term

Blood and Lymphatic System Disorders	Anemia
Cardiac Disorders	Arrhythmia, ventricular arrhythmia, bradycardia, atrioventricular block, cardiac arrest, cardiac disorder, myocardial infarction, extrasystoles, atrial fibrillation, tachycardia, supraventricular tachycardia, ventricular tachycardia
Eye Disorders	Photopsia, visual impairment
Gastrointestinal Disorders	Abdominal pain, diarrhea, nausea, vomiting
General Disorders and Administration Site Conditions	Chills, hyperpyrexia, pain, pyrexia, thirst
Hepatobiliary Disorders	Hepatic function abnormal, hyperbilirubinemia
Investigations	Increased gamma-glutamyl transferase, alanine aminotransferase increased, aspartate aminotransferase increased, increased blood alkaline phosphatase, blood urea increased, electrocardiogram T wave inversion, electrocardiogram QT prolonged
Metabolic and Nutrition Disorders	Acidosis, hyperkalemia, hypoglycemia, hypovolemia, hypernatremia
Nervous System Disorders	Convulsion, dizziness, headache, neuralgia, neuritis, speech disorder
Psychiatric Disorders	Agitation, confusional state, delirium, hallucination, illusion
Renal and Urinary Disorders	Oliguria, polyuria
Respiratory Thoracic and Mediastinal Disorders	Apnea, bronchospasm, dyspnea, hypercapnia, hypoventilation, hypoxia, pulmonary congestion, respiratory acidosis
Skin and Subcutaneous Tissue Disorders	Hyperhidrosis, pruritus, rash, urticaria
Surgical and Medical Procedures	Light anesthesia
Vascular Disorders	Hemorrhage, blood pressure fluctuation, hypertension, hypotension

SYMPTOMS AND TREATMENT OF OVERDOSE

The tolerability of Dexmedetomidine injection was studied in one study in which healthy adult subjects were administered doses at and above the recommended dose of 0.2 to 0.7 mcg/kg/hr. The maximum blood concentration achieved in this study was approximately 13 times the upper boundary of the therapeutic range. The most notable effects observed in two subjects who achieved the highest doses were first degree atrioventricular block and second-degree heart block. No hemodynamic compromise was noted with the atrioventricular block and the heart block resolved spontaneously within one minute. Five adult patients received an overdose of Dexmedetomidine injection in the intensive care unit sedation studies. Two of these patients had no symptoms reported; one patient received a 2 mcg/kg loading dose over 10 minutes (twice the recommended loading dose) and one patient received a maintenance infusion of 0.8 mcg/kg/hr. Two other patients who received a 2 mcg/kg loading dose over 10 minutes, experienced bradycardia and/or hypotension. One patient who received a loading bolus dose of undiluted Dexmedetomidine injection (19.4 mcg/kg), had cardiac arrest from which he was successfully resuscitated.

EFFECT ON ABILITY TO DRIVE AND USE MACHINE

Patients should be advised to refrain from driving or other hazardous tasks for a suitable period of time after receiving dexmedetomidine injection for procedural sedation.

INSTRUCTION FOR USE

Preparation of Solution

Dexmedetomidine injection must be diluted in 0.9% sodium chloride solution to achieve required concentration (4 mcg/mL) prior to administration. Preparation of solutions is the same, whether for the loading dose or maintenance infusion.

Strict aseptic technique must always be maintained during handling of Dexmedetomidine injection.

To prepare the infusion, withdraw 2 mL of Dexmedetomidine injection and add to 48 mL of 0.9% sodium chloride injection to a total of 50 mL. Shake gently to mix well.

Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration, whenever solution and container permit. Do not use if product is discolored or if precipitate matter is present.

Compatibility with Natural Rubber

Compatibility studies have demonstrated the potential for absorption of Dexmedetomidine to some types of natural rubber. Although Dexmedetomidine is dosed to effect, it is advisable to use administration components made with synthetic or coated natural rubber gaskets.

STORAGE CONDITION

Store below 30°C.

After dilution: chemical and physical in-use stability has been demonstrated for 24 hours at 30°C and 2°C – 8°C at 4 mcg/mL in 0.9% Sodium chloride injection.

From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

SHELF LIFE

24 Months

After dilution: chemical and physical in-use stability has been demonstrated for 24 hours at 30°C and 2°C – 8°C at 4 mcg/mL in 0.9% Sodium chloride injection.

From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

PRESENTATIONS

UNIDEX is packed as 2 ml solution filled in 2ml tubular, USP type I clear glass vial, stoppered with 13 mm Teflon coated rubber stopper and sealed with 13 mm Aluminium flipoff seals. Such 25 labelled vials are packed in a carton along with pack insert.

MANUFACTURED BY:

Gland Pharma Limited,
Survey No. 143-148, 150 & 151,
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D.P. Pally, Dundigal Post,
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Medchal – Malkajgiri District,
Hyderabad – 500043,
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PRODUCT REGISTRATION HOLDER & IMPORTER:

UNIMED SDN BHD
No. 53, Jalan Tembaga SD 5/2B, Bandar Sri Damansara,
52200 Kuala Lumpur, Malaysia.

DATE OF REVISION

14.06.2024

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